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The FRAME model for manualization: Developing a flexible and responsive telehealth art therapy approach for military PTSD

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ABSTRACT

Creative arts therapists often express reservations about standardizing therapeutic protocols due to a perceived lack of individualization of care and concerns regarding assumed inflexibility of interventions. To address these concerns, we describe how an interdisciplinary team of art therapists, clinical psychologists, physicians, and Veterans developed an eight-week manualized telehealth art therapy program for military populations with post-traumatic stress disorder (PTSD): Art therapy Remote Treatment for Veterans Experiencing Trauma (ART-VET). This article describes how we designed this manualized intervention to maintain the important individualized, spontaneous, and creative aspects of person-centered art therapy treatment while enhancing treatment fidelity and rigor. ART-VET's flexibility and individualized design paired with systematized clinical decision-making make this a responsive and innovative approach to manualization. The authors identify a framework for flexible art therapy manualization called the Flexible & Responsive Art therapy Manualization for Evidence (FRAME) model. We describe our process of creating ART-VET and plans to test its treatment fidelity and effectiveness using the FRAME model. Recommendations for art therapists seeking to develop flexible, manualized art therapy interventions using the FRAME model are provided.

Background

Manualized therapies identify the core components of a therapy intervention and use standard instructions, often outlined in treatment manuals, for consistent delivery of those components across settings, providers, and patients (Addis et al., 2006; American Psychological Association, 2018). Some mental health providers, including creative arts therapists, welcome the structure of manualized protocols, while others may worry that standardized approaches do not reflect patient needs, oversimplify complex issues, and/or restrict creative expression (Addis et al., 2006; Bryl & Goodill, 2020; Forbat et al., 2015; Millstein et al., 2020). To address these concerns, authors are encouraged to incorporate flexibility within their manuals to allow for individualization, creativity, and to meet individual needs (Addis et al., 2006; Galovski et al., 2024; Kendall et al., 2008; Millstein et al., 2020).

Manualization and the creative arts therapies

The creative arts therapies (CATs) include disciplines of art therapy, music therapy, dance/movement therapy, poetry therapy, drama therapy, and psychodrama, and utilize their respective creative arts modality in psychotherapy and/or rehabilitation (Levy et al., 2025). Art therapy specifically incorporates visual artmaking (e.g., painting, drawing, collaging) into the psychotherapeutic/rehabilitative process (American Art Therapy Association, 2022). Despite the growing understanding of the utility of CATs, the relative lack of a strong evidence base has hindered wider adaptation of these disciplines (Levy et al., 2025). Further specificity for CAT interventions is needed to better

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understand treatment effectiveness and mechanisms of change. Research studies that examine the impact of "the creative arts therapies, " or individual disciplines (such as dance/movement therapy or drama therapy), do not adequately convey what the intervention entails and risk reducing disciplines to techniques. Much as psychologists define and study specific interventions such as Acceptance and Commitment Therapy (ACT) or Cognitive Behavioral Therapy (CBT), creative arts therapists can develop further support for their disciplines by describing and naming specific interventions and the populations for whom they apply. Developing manuals with sufficient specificity and across interventions, settings, and populations can reduce variability and improve the overall quality of patient care. This is particularly important as research findings developed in specific clinical settings and exploring specific interventions may not apply to every clinical context.

Systematizing and standardizing CAT interventions can be difficult due to the need to promote replicability while maintaining individualization, improvisation, and creativity (Bryl & Goodill, 2020). Additionally, prior efforts to standardize CAT research have specified that participants use the same materials and/or follow the same process to reduce variability (Betts & Deaver, 2019), inhibiting the generalizability of research findings to the real-world clinical contexts. Developing manualized CAT interventions that are both replicable with fidelity (or able to be consistently delivered) and grounded in real-world contexts can help to advance research that more closely reflects clinical practice and facilitates clearer operational definitions for clinical practice.

While manualized treatments are not necessarily more effective than non-manualized treatments, their standardization and ease of replicability are primed for research study, particularly in randomized controlled trials (RCTs), which are essential to develop evidence-based treatments (EBTs) (Forbat et al., 2015; Truijens et al., 2018). Further, standardization offers the potential to optimize treatments via better understandings of mechanisms of change. Standardizing CAT interventions and creating manuals for specific conditions facilitates the development and implementation of RCTs to evaluate efficacy of a treatment or intervention (Bryl & Goodill, 2020; Millstein et al., 2020). Using manualized treatments in RCTs would strengthen the evidence-base for these therapies by allowing direct comparison between the results of different investigators. Few such practices have been established in the CATs (Levy et al., 2025).

As this paper describes the development of a manualized art therapy protocol, we explored the existing literature and found three manualized art therapy interventions. First, Manual-based Phenomenological Art Therapy for patients with depression (PATd), created by Blomdahl et al. (2018) uses phenomenological art therapy to treat people with moderate and severe depression. Its efficacy has been examined in an RCT which found that participants who received PATd combined with treatment as usual (TAU) experienced a statistically significant reduction of depressive symptoms (Blomdahl et al., 2022). Second is Haeyen and Heijman's (2020) Compassion Focused Art Therapy, which is described in detail in a peer reviewed article; however, the authors were unable to locate the corresponding treatment manual. Third is Mindfulness-Based Art Therapy (Stull, 2016). Though it is cited in peer reviewed articles by art therapists (Beerse et al., 2020; Van Lith et al., 2021), it was not developed by an art therapist and does not require an art therapist to administer the approach. Although there have been efforts to manualize and create standardized art therapy protocols that can be replicated, the sparse number of manualized art therapy treatments tailored to address limited conditions demonstrates the need for a concerted interventions to further manualize art therapy efforts to address clinical needs (Bachtler & Gratama, 2025).

Project context and rationale

This project grew from a need for art therapy manuals and telehealth protocols for military and Veteran personnel with post-traumatic stress disorder (PTSD). PTSD develops following direct experience of or exposure to life-threatening events, serious injury, and/or sexual violence. It is characterized by intrusive symptoms (e.g., nightmares, distressing memories, and flashbacks), avoidance of trauma-related reminders, negative changes in mood and cognition, hypervigilance, sleep disturbances, and dissociative symptoms (American Psychiatric Association, 2022). While Veteran PTSD prevalence is only slightly above that of civilians at 7%, the lifetime prevalence of PTSD among Veterans who served in Operation Iraqi Freedom (OIF) and Operation Enduring Freedom (OEF) (ranging from 2001 to 2014) is significantly higher at 29% (U.S. Department of Veterans Affairs, 2025a). PTSD can result in functional challenges such as homelessness (Anderson et al., 2023; Carlson et al., 2013), relational distress (Fredman et al., 2022; Gerlock et al., 2013), and increased rates of suicidality (Akbar et al., 2023; Ames et al., 2019). Thus, effective approaches that reduce trauma symptoms, improve functioning, and promote wellbeing are crucial.

There are many approaches to treating PTSD in military populations. An evidence-based therapy refers to a psychotherapy that has been proven effective through rigorous research and is often endorsed by healthcare systems (Department of Veterans Affairs & Department of Defense, 2023). Current clinical practice guidelines across Veteran's Health Agency (VHA) and Defense Health Agency (DHA) recommend the following trauma-focused EBTs for treating PTSD (U.S. Department of Veterans Affairs, 2025b; 2025c): Cognitive Processing Therapy (CPT; Resick et al., 2017), Prolonged Exposure (PE; Foa et al., 2019) and Eye Movement Desensitization and Reprocessing (EMDR; Shapiro, 2018). While these treatments have large effect sizes (Lee et al., 2016; Watts et al., 2013), many people struggle to complete them due to associated distress and therefore may not receive their full benefits (Edwards-Stewart et al., 2021; Najavits, 2015). Some of the critiques of current EBTs in the VA include their negative emotional intensity (Doran et al., 2021; Steenkamp et al., 2020) and lack of personalization (Doran et al., 2021).

Art therapy is generally well-regarded in military and Veteran populations (Kaimal et al., 2019), and personally tailored art therapy interventions for populations with PTSD may be more tolerable than other treatment approaches, helping to decrease treatment avoidance (Lobban & Murphy, 2019). Indeed, one small study reported a 40% dropout of CPT compared to 0% when combined with art therapy (Campbell et al., 2016). However, CAT programs vary greatly between medical centers, and within the VHA and DHA these services typically take place within larger systems of care where it can be challenging to distinguish their unique contributions (Department of Veterans Affairs & Department of Defense, 2023). Additionally, creative arts therapists may or may not be allowed to incorporate trauma-focused CAT interventions into their clinical work because many are not classified as mental health providers at these sites. Having evidence-based treatments that are specific to the CATs can clarify creative arts therapists' scope of practice and allow therapists to practice in ways that are reflective of their experience and training both in the VHA/DHA and in other health systems.

In sum, well-designed manuals need not restrict clinical artistry. Rather, they invite us to clarify the core elements of effective practice, strengthen fidelity, and build a consistent foundation for the field. By articulating essential components and intentional variations, standardized processes can support both clinical autonomy and the evidence base needed to secure art therapy's place in healthcare systems. This paper describes a novel model to guide future art therapy manualization efforts that support the individualization of services while standardizing clinical decision making. We present our process of creating a flexible, manualized program to treat military and veteran PTSD as an example of its use.

The FRAME model for flexible art therapy manualization

In this paper, we describe the development of a manualized art therapy protocol that addresses the growing need for standardization in the CATs while preserving their inherent creativity and person-centered

practices. The Art therapy Remote Treatment for Veterans Experiencing Trauma (ART-VET) protocol approaches the standardization of a telehealth-based art therapy protocol for Veterans with PTSD as a set of decisions that therapists can make in a consistent way, allowing for the treatment fidelity necessary for rigorous study in RCTs. It aims to standardize the core elements of treatment while preserving the therapist's flexibility and ensuring personalized care. For the purposes of ART-VET and for this article, the authors use the term "Veterans" to include both Veterans and active-duty military personnel who receive ART-VET.

In reviewing our process for developing the manual, the authors were able to identify clear but fluid steps that were followed, and that we believe could be applied by other (art) therapists seeking to manualize their process. The Flexible & Responsive Art therapy Manualization for Evidence (FRAME) model introduces those steps, and we demonstrate how they were implemented in our process of developing the ART-VET manuals, which are currently being tested for fidelity across providers. We offer the FRAME model as a framework that has been applied to art therapy manualization and offers potential for application to other CATs and mental health disciplines (Fig. 1).

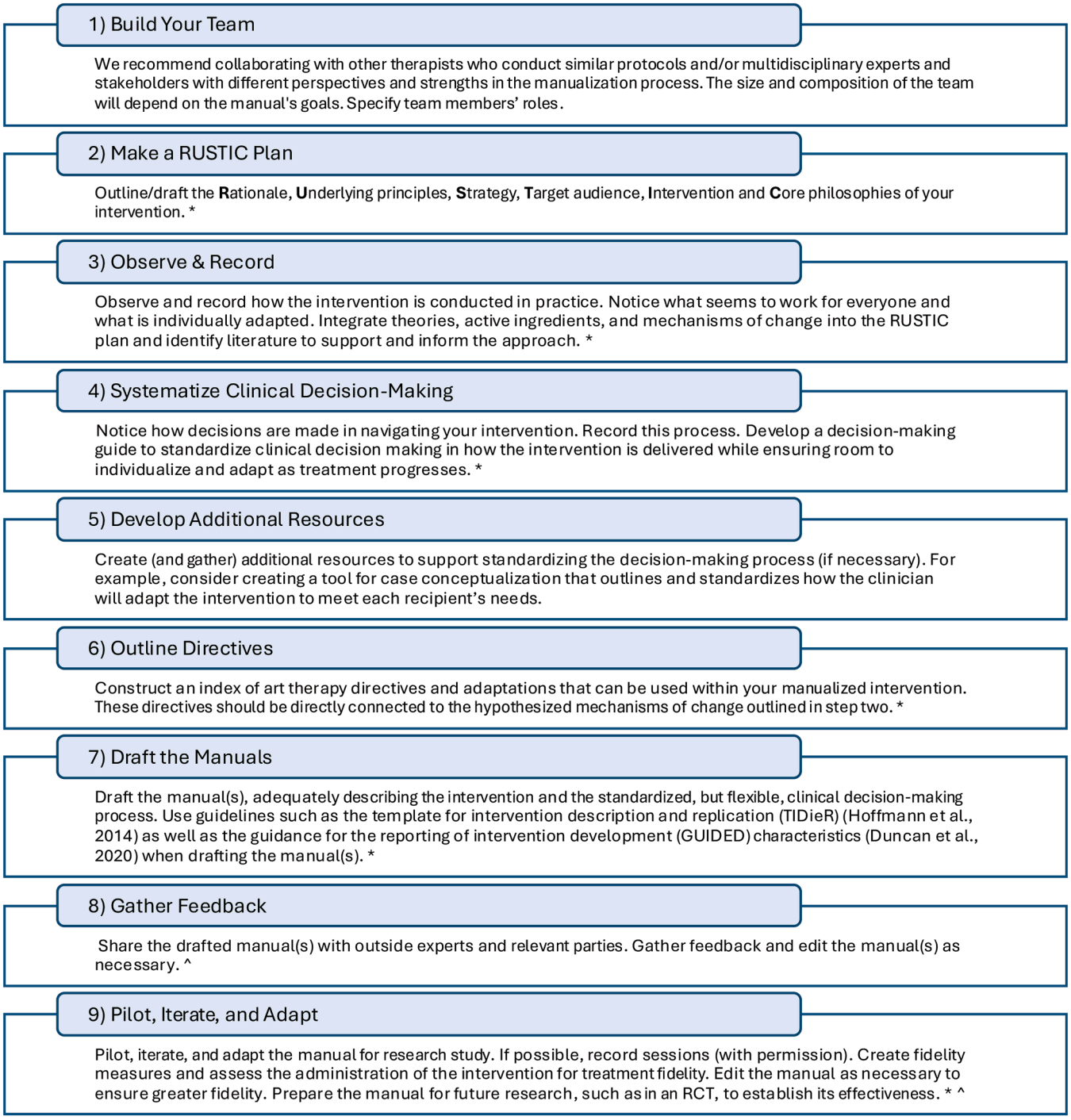


Fig. 1. The FRAME Model for Flexible Art Therapy Manualization. Note. The FRAME model was developed by H. Spooner and K. S. Stickley (2026). The steps with an asterisk (*) align with Carroll and Nuro's (2002) descriptions of Stage I manual development. The steps with a caret (^) align with Bryl and Goodill's (2020) steps in developing their manualized dance/movement therapy protocol.

FRAME case example: developing ART-VET

ART-VET is based on a semi-structured telehealth art therapy protocol that was initially developed in 2014 for use with rural Veterans (Levy et al., 2018; Spooner et al., 2019). Telehealth was chosen as the delivery method due to its ability to support Veterans in overcoming treatment barriers related to location and/or disability (Doran et al., 2021; Levy et al., 2018). Although telehealth offers increased access to psychotherapy for such Veterans, potential drawbacks of telehealth include privacy and safety concerns as well as technology barriers, which could limit some Veterans' participation (Jones et al., 2020; Jones et al., 2026). Telehealth also limits art therapists' ability to see both the Veteran's facial expressions and their art making process simultaneously. Since this intervention was delivered by a single therapist, many standardized elements of the intervention were conducted intuitively and were not explicitly pre-defined.

This protocol was used as the foundation for a previous research study (Jones et al., 2026; Valldejuli et al., 2025). The study team later determined that further standardizing and manualizing the protocol would allow for the development of a stronger research base and comparisons across sessions and therapists. Through ART-VET, we aimed to develop an intervention that would support Veterans' recovery from PTSD while being consistently delivered, lending itself to rigorous study, and promoting co-creation and individualization of the therapeutic process among art therapists and Veterans.

We will use the FRAME model to describe our process of creating the ART-VET intervention.

1) Build your team

Building your team means finding others who can collaborate on the manualization process. The size and composition of the team will depend on the manual's goals. A manual that is designed to create consistency within an individual program may be developed in collaboration with a peer or supervisor; while a manual that is developed for research will require a larger and more diverse team.

ART-VET is supported by a large multi-disciplinary team. This team was assembled to support research and fidelity testing across multiple sites and consists of three art therapists/art therapy researchers (HS, KSS, and GK), a neuropsychologist (JW), a neuroscientist (DL), a clinical health psychologist and Veteran with experience in creating psychotherapeutic treatment manuals (JG), a physiatrist (C. Levy), two graduate assistants (LJ & KS), and two Veterans with educational backgrounds in psychology and related disciplines (C. Lee & SB). The team met weekly and collaborated throughout the process. We found that having people on our team who were unfamiliar with art therapy led us to more critically evaluate and clearly explain this work. Additionally, having a team that knew the population intimately, and in many cases belonged to the population themselves, led us to more deeply consider how the protocol worked for the Veterans we serve.

2) Make a RUSTIC plan

Making a RUSTIC Plan refers to identifying the Rationale, Underlying principles, Strategy, Target audience, Intervention, and Core philosophies that inform the manualized art therapy intervention. "RUSTIC" reflects that this is a rough, initial foundation for the manualization efforts, which will be refined as the therapist applies it to future sessions, notes what is consistent or inconsistent, and records their observations.

Rationale

Rationale refers to the aim of the manual, the factors that shape the intervention, the need and desired outcomes of the intervention, and the patterns and observations that have informed the intervention over time (assuming that manualization efforts are rooted in clinical practice).

The rationale for developing ART-VET was shaped by its focus on rural veterans and programmatic need for a time-limited intervention, resulting in a short-term telehealth protocol. The goals were to improve access to art therapy services for Veterans with PTSD and to support Veterans in reducing the impacts of PTSD on their daily life. Some of HS's observations that further shaped the intervention's rationale over time included that Veterans participating in home telehealth engaged more frequently and found greater benefit from artmaking between sessions, often reported greater comfort while processing traumatic content in the privacy and comfort of home, found some of the highest impact sessions to be those that focused on understanding their trauma experience and integrating it with who they are and who they want to become, and showed greater investment and made more progress towards treatment goals when they actively participated in decision making.

Underlying principles

Underlying principles refers to the theoretical foundations, mechanisms of change, and active ingredients that relate to and inform the protocol. Identifying the manualization effort's underlying principles allows clinicians to integrate their program's rationale with the available supporting evidence.

ART-VET was informed by evidence on factors contributing to art therapy's effectiveness, both generally and specific to PTSD recovery. In a scoping review, de Witte et al. (2021) identified significant factors of CATs as "embodiment," "concretization," "symbolism," and "metaphors" (p. 17). These factors are distinct from other psychotherapies. Qualitative research seems to support such factors as significant to recipients of art therapy (Ben-David et al., 2025). Specific to PTSD recovery in military populations, active components of art therapy are "concretization, symbolism, and metaphor," "active and creative art engagement," "emotion processing and regulation," "perspective taking and reframing," and the therapeutic relationship (Malhotra et al., 2024, p. 4–7).

We also aimed to design an intervention that was aligned with proposed mechanisms of change identified in other evidenced based trauma treatments, such as addressing trauma-related cognitions and supporting emotional regulation, while addressing some of the critiques related to a lack of individualization, emotional intensity and titration of traumatic content that may lead to high attrition rates.

Strategy

Strategy determines how the protocol will be designed and implemented. Therapists must identify the "active ingredients" (or necessary elements to support therapeutic change) informed by the rationale and underlying principles. Therapists should sequence the manualized protocol (e.g., what intervention elements will be provided at what times), determine appropriate dosage of the intervention (e.g., how much of the therapeutic intervention is needed), and incorporate individualized decision making into the larger manualized structure.

To determine our ART-VET strategy, we compared the current intervention to existing psychotherapy treatment manuals. Based on JG's previous experience in manualization efforts, the *Cognitive-Behavioral Therapy for Insomnia in the Military: Therapist Guide* by Taylor et al. (2019) provided a basic structure on which to model the art therapy manual. HS and KSS then compared the existing intervention to the EBTs for PTSD recognized by the VHA and DHA: CPT, PE, and EMDR. These therapies broadly involve three phases: 1. establishing therapeutic rapport, psychoeducation, safety, and stability; 2. processing the traumatic experience(s); and 3. re-telling these experiences until they are less distressing. These phases aligned well with the intervention as previously developed by HS, and thus offering further support for the authors' existing protocol.

ART-VET consists of eight 50–60-minute telehealth sessions organized into three stages of treatment. In the first stage (sessions 1 and 2) with the support of the art therapist, the Veteran completes an initial assessment, identifies treatment goals, and explores their wellness, identifying areas of strength and areas that need attention. This stage emphasizes the therapeutic relationship and co-construction of the sessions to come. The second stage (sessions 3–6) provides a Clinical Decision-Making Guide (CDMG) to support the art therapist in navigating the five potential session topics of this stage: 1) Psychosocial Safety & Self-Regulation, 2) Veteran Led Session, 3) Trauma Tolerance & Processing, 4) Integration, and 5) Flex Session. Necessary steps in Stage 2 include establishing psychosocial safety and self-regulation (this may or may not require its own session) and focusing on trauma tolerance, processing, and integration. The final stage of ART-VET (sessions 7 and 8) emphasizes maintenance planning, post-therapy assessment, and closing of the experience.

In comparing the current protocol to those in the EBTs, the authors identified that processing of traumatic content in art therapy may occur symbolically, metaphorically, or non-verbally. This means that the degree to which trauma processing occurs may not be immediately evident in the way that it may be in more concrete verbal therapies. Trauma Tolerance and Trauma Processing are related processes that we conceptualize as existing on a continuum. Similar to entering a body of water, the depth to which one enters may not be immediately evident (Fig. 2). We recommend that the art therapist find a point of entry along this continuum based on Veteran's ability to regulate, readiness, thinking style, and connection to symbols and metaphors.

Recommended directives for this phase are those that specifically address trauma experiences (e.g., memories) as well as those that are more metaphoric and allow for symbolic and nonverbal expression. When therapists identify that deeper processing is safe and would provide therapeutic benefit, they are asked to encourage deeper processing of traumatic content through Socratic questioning or changing the art product in some way. Higher levels of distress may indicate that the Veteran is at the edge of their trauma tolerance, and deeper processing is not advised. However, the inability to engage in extended verbal processing does not necessarily indicate that deeper processing has not occurred.

We have found that some metaphors commonly used in art therapy are a valuable way to provide both invitations to explore traumatic content in depth and have safe distance from traumatic content when needed. These include masks, boxes, bridges, and doorways (among others). Some of these metaphors may also serve the dual purpose of processing the trauma experience and offering a point of entry for how that experience can be integrated in one's current life. For example, depicting trauma on one side of a bridge and illustrating where someone is moving towards on the other side.

Target audience

Identifying the target audience in the RUSTIC process refers to indicating for whom the manualized art therapy intervention is being created.

The target audience for ART-VET is active-duty military personnel and Veterans with PTSD. However, ART-VET might not be appropriate for every Veteran with PTSD. Art therapists should make ethical considerations in determining whether or not participating in ART-VET might be beneficial to the Veteran. Telehealth is considered safe for most Veterans and can improve access to care, but art therapists should consider several factors such as: physical or cognitive issues that would make it difficult for the Veteran to use the technology or art materials independently, a lack of privacy in the home, difficulty with reality testing, social isolation, or the need to use additional art materials that may be available in an art therapy clinic, among others. Art therapists should use their best clinical judgment in deciding if ART-VET's telehealth-format is best suited for each Veteran.

Intervention

In the FRAME model, "Intervention" refers to using the first four elements of the RUSTIC Plan to determine the structure of the protocol's sessions. These structural components could relate to the session timeline (e.g., how much time will be spent on artmaking or processing), goals, activities, materials, homework, and possible adaptations of the intervention. This step can be iterative and evolve throughout the manualization process, as art therapists may consider refining their approach as they develop their intervention.

For ART-VET, sessions 1 and 8 do not involve artmaking, but are conversational. In the first session, the art therapist and Veteran get to know each other and consider how future sessions will incorporate artmaking. The last session is designed to facilitate a discussion about

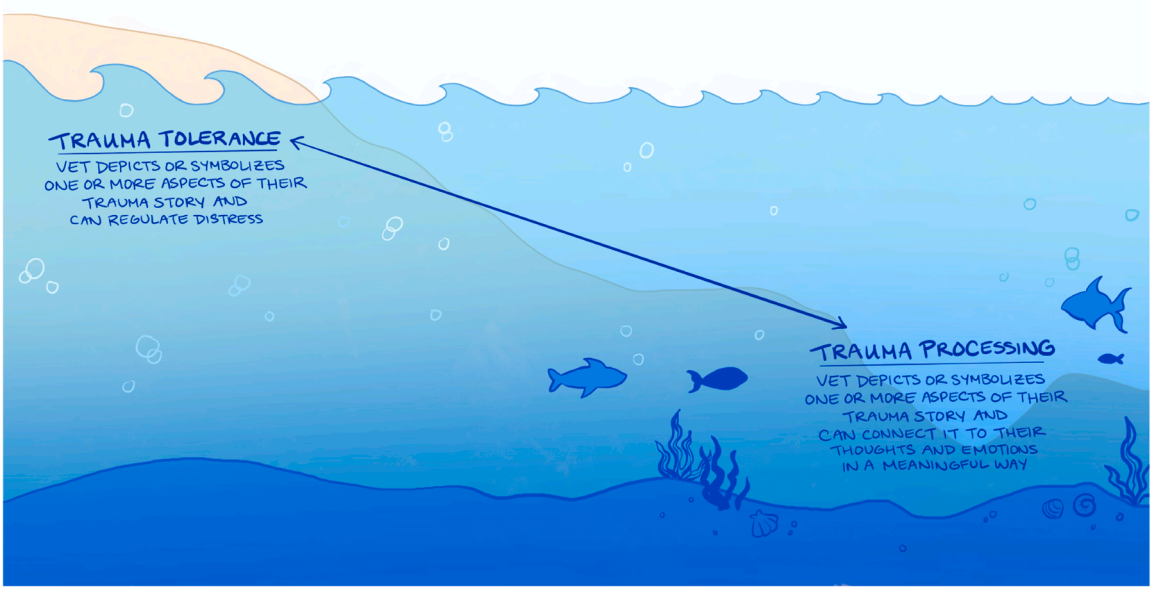


Fig. 2. ART-VET Trauma Tolerance & Trauma Processing Continuum. Note. Illustrated by K. S. Stickley. Retrieved from the unpublished ART-VET Art Therapist Manual (Spooner et al., 2025).

the Veteran's experience in ART-VET, a review of their artwork, and discussion of maintenance. Sessions 2 through 7 involve artmaking, and all follow the same basic session structure. Each session begins with a check in where the Veteran can share updates and artwork they may have completed between sessions. The art therapist will then provide an introduction to the session's topic and present or co-create a prompt for art making. The Veteran will engage in art making, and after about 25 minutes of art making time, the art therapist will facilitate a discussion of the artwork. Suggested language and adaptions for these discussions are outlined in the manual and include discussing the process of artmaking, the Veteran's emotional reaction, the meaning of their work, and imagining change.

We created a Guide & Workbook for the Veteran to use as they complete the ART-VET protocol, which has various worksheets and arts engagement activities. These resources are designed to function as homework activities to supplement material provided in session.

Core philosophies

The last component of the RUSTIC model is Core Philosophies, which are the broad values or beliefs that guide the manualization approach.

In developing ART-VET, our core philosophies were that creativity is a health behavior; that treatment should be personalized and tailored to each Veteran's needs; and that art therapy manualization should balance creativity with standardization in its application.

As a team of art therapists and other allied health professionals, we recognize that the creative process of art making is a behavior that supports health and wellbeing (Fancourt & Steptoe, 2019; Sonke et al., 2025a; Sonke et al., 2025b) that can and should be accessed outside of formal art therapy sessions. As such, we developed ART-VET to be a comprehensive treatment protocol that not only engages the Veteran in art making during ART-VET sessions, but that also promotes creative self-expression outside of sessions. In practice, discussions about making creativity a part of daily life are regularly integrated into ART-VET sessions (and the Guide & Workbook) and are key in maintenance planning.

Another core philosophy that guided our ART-VET manualization efforts is that we believe that art therapy should be tailored to meet the needs of each Veteran. We conceptualize this philosophy to be aligned with the concepts of person-centered care (Murphy & Joseph, 2016) and the VA's Whole Health initiative (U.S. Department of Veterans Affairs, 2025d), and designed ART-VET in a way that prioritizes Veteran agency and participation in their treatment. ART-VET prioritizes each Veteran's unique goals, wellness strategies, and recovery journey, allowing the Veteran and art therapist to individualize and co-construct their process within the manualized structure.

Our final guiding principle is to promote both flexibility and creativity within the intervention *and* standardization of content and context so that ART-VET can be individualized while being reliably administered to military populations with PTSD. We aimed to do this by identifying and standardizing the key decision-making processes in administering this treatment while building in elements (such as case conceptualization tools) that ensure flexible, responsive care.

3) Observe and record

The Observe & Record step of the FRAME model refers to the therapist taking the time to deeply notice and document what they do when they deliver the intervention of interest. This description should include elements of the intervention that occur every time it is delivered, what the decision points are, and when and how adaptations are made. This step also involves integrating theories, active ingredients, and mechanisms of change into the RUSTIC plan and supporting the work through what has been found in the peer-reviewed literature.

In developing ART-VET, the primary authors of the manual were the therapist who developed the intervention (HS) and another art therapist (KSS). They met weekly to discuss and draft both the observable steps of each session and the underlying decisions that the therapist made at each stage. Over time, HS began to identify which core elements of the protocol were important for all Veterans to receive and which could be adapted to individual needs. This flexibility also aligned well with the responsive nature of the semi-structured telehealth approach. Having never conducted the protocol herself, KSS identified what was missing from the instructions at each stage and contributed to deeper exploration around the art therapist's underlying decisions that informed the treatment trajectory. This outside perspective was beneficial in highlighting assumptions that were not explicitly stated. Continued work on a previous study (Jones et al., 2026; Valldejuli et al., 2025) also allowed HS to compare the steps that were outlined to what took place in practice and note any discrepancies between the treatment sessions and what was drafted.

4) Systematize clinical decision making

Once the process is recorded, the next step is to develop a system that allows others to conduct therapy sessions, and navigate decision points, in a consistent way while still promoting flexibility and individualization. This can be a visual model, flow chart, decision tree, or simply a step-by-step process that clearly describes these decision points at each stage. The model should prioritize simplicity so that other therapists can understand the steps and follow the process with fidelity.

Throughout the process of drafting and testing the ART-VET protocol, we developed resources to support and concretize the clinical decision-making processes of the art therapist to support consistent delivery of ART-VET material. These included a Clinical Decision-Making Guide (CDMG), tools for case conceptualization, and written steps for how to administer specific art therapy directives. These tools, along with the original checklists, became the foundation upon which the manual was built.

ART-VET consists of eight sessions, of which four session topics are offered to everyone, and four session topics are customized. The CDMG (Fig. 3) is a visual model that supports the art therapist in determining which of the five session topics to administer in sessions 3 through 6. These topics include: 1) Psychosocial Safety & Self-Regulation, 2) Veteran Led Session, 3) Trauma Tolerance & Processing, 4) Integration, and 5) Flex Session. Each Veteran may move through these topics in different ways, skipping topics that are less relevant or repeating topics that warrant further exploration. Each Veteran will experience at least one session dedicated to Trauma Tolerance & Processing and one session exploring Integration. Like a decision tree, the CDMG provides guidance on how to consider whether each topic area is necessary to administer and in what order to facilitate these sessions by asking the art therapist a series of "yes/no" questions. This in turn supports the art therapist in individualizing the ART-VET protocol to meet the needs and goals of each Veteran.

As outlined in step 9 of the FRAME model, ART-VET will be piloted in an upcoming research study where the CDMG's utility and clinical application will be examined. It may be further refined based on feedback from the art therapists administering ART-VET.

5) Develop additional resources

The next step of the FRAME model is to develop additional resources to support the clinical decision-making process. This step might also involve gathering resources that already exist to supplement protocol materials.

We created a case conceptualization tool to help clarify the clinical decisions being made within the ART-VET structure and understand how to adapt the structure to meet the needs of Veterans. This tool is a worksheet that asks the art therapist to consider the Veteran's goals, identity, access to and interests in specific art media, and available evidence when choosing interventions for each session. We encourage the art therapist to continually use and update this resource throughout ART-VET sessions for

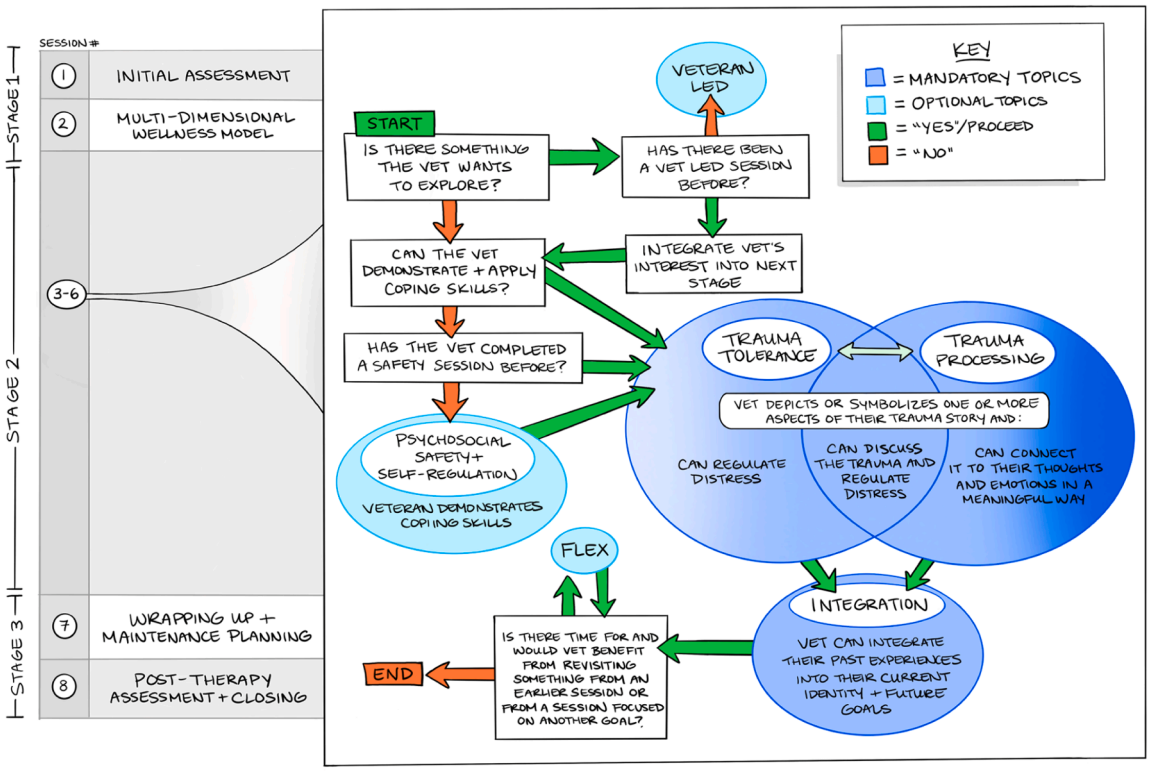


Fig. 3. ART-VET Clinical Decision-Making Guide (CDMG) illustrated by K. S. Stickley. Note. The CDMG is being piloted, and its content may change in future iterations before finalization. Retrieved and edited from the unpublished ART-VET Art Therapist Manual (Spooner et al., 2025).

each Veteran so that they are regularly thinking of adaptations to the protocol that can meet the Veteran's individualized needs. In this stage, we also gathered resources that were already integrated into our protocol, such as tools for safety assessment and safety planning.

6) Outline directives

The next step of the FRAME model is to outline art therapy directives that will be used in the protocol. These directives are the specific prompts that target the issues being addressed by the manualized protocol. We recommend choosing and/or creating directives that are directly connected to the hypothesized mechanisms of change identified in step two of the manual development process.

We identified ten art therapy directives that could be integrated into ART-VET sessions. These directives align with the active components that support change in art therapy for military populations with PTSD: "concretization, symbolism, and metaphor," "active and creative art engagement," "emotion processing and regulation," "perspective taking and reframing," and the therapeutic relationship (Malhotra et al., 2024, p. 4–7). We created a one-page write-up for each directive which includes how the directive aligns with session topics and Veteran goals, step-by-step instructions, recommended art media, suggested adaptations, and references/resources relating to the development of the directive.

The ten directives were identified, refined, and adapted for Veteran use through the team's significant clinical experience in working with military populations. One directive called "Doorway" was previously developed by HS in her art therapy work with Veterans. Using the metaphor of a doorway, this mixed media directive prompts Veterans to reflect on transitions in their life and depict where they came from, where they are now, and where they would like to go. We frame this directive as being supportive of Trauma Tolerance & Processing and/or Integration.

Although we provide a list of recommended art therapy directives, we also encourage art therapists to incorporate directives from their practice so long as they align with ART-VET content. We also encourage art therapists to allow Veteran's media interests and ideas to shape art therapy directives and for art therapists and Veterans to co-create individualized art directives throughout the protocol.

7) Draft the manuals

In writing and reporting ART-VET manuals, the authors adhered to the template for intervention description and replication (TIDieR; Hoffmann et al., 2014) as well as the guidance for the reporting of intervention development (GUIDED) characteristics (Duncan et al., 2020) to promote replicability of our protocol among other art therapists.

ART-VET consists of three different targeted manuals: 1) the Art Therapist Manual for art therapists, 2) the Guide & Workbook for Veterans, and 3) the Technical Manual for researchers studying the protocol. These manuals were written iteratively, and HS and KSS often met biweekly for co-writing sessions.

ART-VET Art Therapist Manual

The Art Therapist Manual is designed for the art therapist to use throughout their delivery of ART-VET. It has three main sections: Part I: Background & Context describes the intervention's theoretical underpinnings, necessary art therapist qualifications and characteristics, telehealth troubleshooting, safety and ethical considerations, and an overview of the materials. Part II: Session Protocol Guide provides step by step instructions on delivering the intervention (including how to use the CDMG). Part III: Appendices provides additional resources for the art therapist, including the list of the art therapy directives and tools for case conceptualization. Resources outlined in the Appendices are as follows (Table 1):

Other key elements of the Art Therapist Manual are the session guides which provide introductions to each session topic, references to the ART-VET Guide & Workbook, and a step-by-step outline of each session.

Table 1
ART-VET appendices.

Appendix Component	Description
Individualized Session Planning to Support the Veteran's Personal Goals	• Provides guidance on how to align Veterans' goals with art therapy media, processes, and directives, making clinical recommendations grounded in research. • Provides a tool for art therapy case conceptualization which asks the art therapist to consider the Veteran's goals, identity, access to and interests in specific art media, and available evidence when choosing interventions for each session.
Art Therapy Directives	• Outlines ten art therapy directives that can be integrated into ART-VET sessions, including what session topics might align with the directive, how the directive might align with Veteran goals, recommended art media, and ways the directive can be adapted and individualized to the Veteran.
Session Checklists	• Provides a copy of the CDMG, a space for clinical decision making to be documented, and a checklist for each ART-VET session that includes the key elements that need to be administered.

ART-VET Guide and Workbook

ART-VET includes a physical Guide & Workbook for the Veteran to use throughout the ART-VET intervention and beyond. It allows the Veteran to follow along with the protocol at home and provides structured tools for creative explorations outside of sessions, underscoring our belief that making art is a health behavior. The Guide & Workbook will be mailed to Veterans for ART-VET treatment. To create this resource, HS and KSS collaborated with C. Lee and SB (Veterans) to develop background information, psychoeducation, and art education exercises, while writing in a familiar tone. KSS created graphics and illustrations throughout to use a different modality to communicate information. The team worked with a graphic designer to create a visually appealing, durable, and accessible final product. Exemplar spreads of the final workbook are displayed in Fig. 4.

The Guide & Workbook has three sections: Section 1: Introduction, Section 2: Session Topics, and Section 3: Your Art Space. The introductory section of the Guide & Workbook outlines basic information about what art therapy is, why it is helpful in processing trauma, and expectations for telehealth sessions. It also provides a list of art materials the Veteran will receive when participating in ART-VET and offers tips and tricks for working with those materials.

Section 2 outlines five main topics that ART-VET sessions will follow. These topics align with the sessions outlined in the CDMG. Some topics are optional, and some are mandatory, and these topics may be covered at different timepoints based on the art therapist's discretion using the CDMG. These topics are titled: Multidimensional Wellness Model; Building Safety & Coping Skills; Working Through Trauma; Integration: Putting it All Together; and Maintenance Planning. Each topic has at least two pages of relevant psychoeducation which can complement and reinforce content from sessions and at least two corresponding worksheets for the Veteran to use during and/or between sessions. These worksheets provide additional space to reflect on the psychoeducational content and/or prepare for subsequent sessions.

Section 3: Your Art Space is the last section of the Guide & Workbook. First, we provide information on how arts engagement can be a part of daily life. Then, we include art education worksheets on topics such as the elements of art, color mixing, value and shading, perspective, and drawing human figures, with the idea that the Veteran could follow their interests and build artistic skills outside of ART-VET sessions, as we recognized the time constraints associated with this eight-session intervention. We expect that these resources will provide Veterans with opportunities to reflect and create between sessions, reinforce concepts explored during sessions, and serve as a transitional resource the Veteran can continue to use following the completion of ART-VET sessions.

ART-VET Technical Manual

The Technical Manual is designed for use by other researchers who would like to replicate our future research study by providing detailed guidance on assessment, monitoring, and outcome evaluation for the ART-VET protocol. Its primary purpose is to support both clinical decision-making and research rigor by outlining a standardized assessment framework, as provided in other evidence-based psychotherapy practices. The manual details a comprehensive battery of validated self-report measures and qualitative assessments administered at defined, clinically relevant points across the treatment timeline. These instruments span multiple domains relevant to Veterans with trauma-related concerns, including psychiatric symptoms, trauma coping, sleep and dream functioning, dispositional factors, therapeutic alliance, and participant engagement. By organizing measures by functional domain, the manual facilitates a multidimensional understanding of Veteran experiences and treatment trajectories. In addition, the manual provides an overview of each instrument's purpose, scoring procedures, clinical cutoffs when applicable, and psychometric properties, supporting consistent implementation, reproducibility, and comparability across clinicians and sites. The manual also provides researchers with a tool to track how art therapists are using the CDMG so that its effectiveness can be further evaluated.

8) Gather feedback

The next step of the FRAME model is to gather feedback on the manualized materials.

Throughout the process of manualization, the ART-VET team regularly provided feedback to the lead authors (HS and KSS) in weekly meetings. However, after the manuals were written, the entire team read and reviewed the materials and provided specific feedback on the consistency of the language, clarifications concerning standardizing clinical decision making, as well as specific adjustments to the protocol based on clinical knowledge and lived experience.

The manuals were then reviewed by three outside consultants: two doctoral art therapists with extensive military treatment experience and one psychiatrist who is a Veteran with extensive experience treating military populations. Their feedback was integrated to further strengthen the overall protocol and ensure clarity.

9) Pilot, iterate, and adapt

After the manual has been drafted, reviewed by relevant experts, and adjusted accordingly, the final steps of the FRAME model are to pilot, iterate, and adapt the manual in a research study.

Once the entire team and expert reviewers provided feedback on the manual materials and the feedback was integrated into the manuals, HS then conducted the entire protocol with a mock participant (also a Veteran and retired military clinical psychologist). These sessions were recorded and assessed for fidelity to the protocol. Piloting the manual in this way allowed for HS and the team to recognize aspects of the protocol/manuals that needed to be further refined as well as possible logistical challenges.

The next stage of this project will be a mixed methods (Wasti et al., 2022) study of Veterans experiencing PTSD to gain feedback on the intervention, assess whether ART-VET can be delivered with fidelity, and gauge whether it effectively promotes PTSD recovery. The study will be conducted in a phased approach: two art therapists will work with

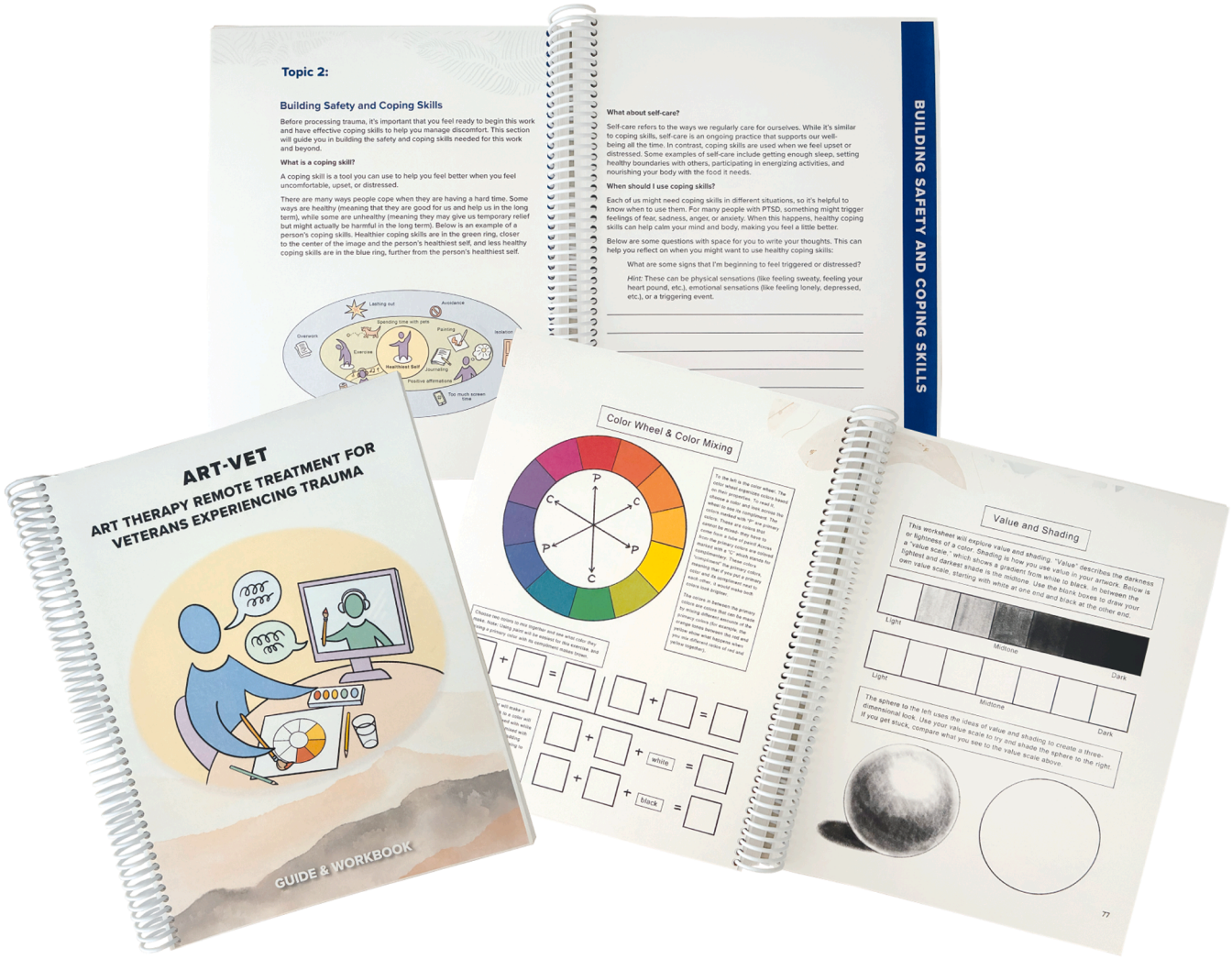


Fig. 4. Spreads from the ART-VET Guide & Workbook. Note. Retrieved from the unpublished ART-VET Guide & Workbook (Stickley et al., 2025).

five Veterans each; feedback will be gathered to inform modifications of the protocol and manuals. Once the materials are updated, the revised materials will be delivered by two additional art therapists who will each see five Veterans. Final feedback from that cohort will then be integrated. Prior to implementing ART-VET, art therapists will receive online training in administering the intervention. Effectiveness will be evaluated pre- and post-ART-VET treatment by assessing PTSD symptoms, quality of life, and other psychosocial factors. Qualitative data will include artwork created in sessions, post-therapy interviews with Veterans, session notes, and interviews with the art therapists.

Central in the creation and establishment of psychotherapeutic interventions is the concept of fidelity, which is the objective assessment of a therapist's adherence to the treatment protocol, helping to ensure that treatments are delivered with integrity, effectiveness, and ethical care (Miller & Rollnick, 2014). In the field of art therapy, attention to fidelity supports consistency in the application of interventions across therapists and settings, allowing successful therapeutic approaches to be reliably replicated with benefit to clinical research and ethical practice. This consistency is essential for the establishment of evidence-based practices and furthering the credibility and legitimacy of art therapy practice by further understanding the mechanisms that underlie therapeutic change. This knowledge, in turn, informs ongoing treatment development, allowing the field to evolve while remaining grounded in evidence-based practice.

In addition to its role in research, fidelity is closely tied to ethical practice in art therapy. Maintaining fidelity helps ensure that therapists uphold client well-being, autonomy, and cultural responsiveness within treatment delivery. By adhering to agreed-upon therapeutic principles and procedures, fidelity supports the creation of safe and trustworthy therapeutic environments, strengthening collaboration between clients and therapists. Encouraging treatment fidelity can take many forms: the development of treatment manuals, therapist training, observation and fidelity monitoring, fidelity assessment tools, and ongoing supervision with feedback (Hildebrandt et al., 2012; Sanetti et al., 2021). Fidelity is best implemented through comprehensive processes that include training, monitoring, and feedback. At the same time, fidelity frameworks must allow for flexibility within manualized treatments such that therapists can respond appropriately to diverse clinical presentations and individual client needs (Kendall & Frank, 2018).

Our approach to treatment fidelity will include weekly supervision meetings with the participating art therapists and the research team as well as objective assessment via video recording. The supervision meetings will provide structured opportunities for therapists to reflect on session experiences, discuss challenges and successes related to manual implementation, and ask questions. In these meetings, we aim to learn more from the participant art therapists about any factors of ART-VET that might contribute to deviations from the manualized approach. These pragmatic clinical implementation data will inform subsequent iterations of the manuals and training of art therapists delivering ART-VET.

In addition, each art therapy session will be recorded via a secure telehealth platform (with participant informed consent). These videos will then be reviewed and assessed by graduate students involved in the research study to document how well the therapist adhered to the ART-VET protocol. To create these fidelity assessments, LJ met with HS and KSS to identify the key ingredients or active components of ART-VET. With critical discussion, HS and KSS were able to outline the key elements of each session based on the session's objectives, interventions, and flow. LJ then took these elements and created an assessment for each session, which included "Yes/No" and "No/Partially/Completely" questions with a final treatment adherence percentage which will be calculated for each session by the session reviewer. Sample questions from the initial session's checklist include, "Did the art therapist at least once reflect back what the Veteran said?" and "Did the art therapist guide the Veteran in identifying three things they would like to see as a result of art therapy?" Such an assessment tool will allow for the objective assessment of whether ART-VET can be delivered with fidelity in its current form. Through this future study, we aim to determine whether the ART-VET protocol supports Veterans' PTSD recovery, whether different art therapy providers can consistently deliver the protocol, and how the current materials can be adjusted to further strengthen its effectiveness and delivery.

Discussion

ART-VET was created as a short-term telehealth intervention to address PTSD in military populations that can be implemented with fidelity. In developing ART-VET, we identified important considerations to create art therapy manuals that promote standardization and replicability while supporting individualized needs, providing flexibility and encouraging the creativity necessary in art therapy treatment. These elements informed our conceptualization of the FRAME model for flexible and responsive art therapy manualization.

In accordance with recommendations to create responsive and flexible manuals that can be implemented with fidelity and bridge gaps between clinicians and researchers, we have rooted ART-VET in our clinical (telehealth) art therapy practice with Veterans and service members (Forbat et al., 2015; Westen, 2002). We created a protocol that has boundaries and treatment parameters but is responsive to each Veterans' needs and allows for the Veteran and art therapist to co-construct their individualized process (Kendall et al., 2008) while clarifying the "active ingredients" of the approach (Carroll & Nuro, 2002, p. 404). With the development of a case conceptualization tool, we created a way for the art therapists to constantly consider the Veteran's unique experiences and background in conceptualizing their treatment trajectory, consistent with recommendations by Galovski et al. (2024) to consider case formulation in flexible, responsive manualized psychotherapy for PTSD. We have also designed the CDMG as a core tool that can help offer guidance and direction during key decision-points of treatment, an essential component of "clinician--friendly" manuals (Carroll & Nuro, 2002). Finally, we have provided "checklists" for the therapists to use during each session to monitor their delivery of the key components of ART-VET (Carroll & Nuro, 2002).

Based on our experience creating the ART-VET protocol, we developed the FRAME model for art therapists to construct manualized protocols that are both standardized and flexible. Of note, this framework does not include rigorous testing of the manualized materials, as will happen with ART-VET in future study. Rather, it aims to describe how the creative and responsive nature of art therapy practice can be translated in a standardized and replicable way. Providing guidance on the clinical decisions made in the intervention is key to promoting this standardization and flexibility, and should be integrated with guidance from other disciplines, such Carroll & Nuro's (2002) three-stage approach to manual development or Bryl & Goodill's (2020) manualized dance/movement therapy protocol, both of which have steps that align with the FRAME model. Future inquiry is warranted as to whether our framework, one that frames manualization as a flexible yet standardized decision-making process, can be applied to other CAT modalities, such as music therapy and dance/movement therapy and non-CAT psychotherapies. Further inquiry can also determine whether the steps of the FRAME model can be used from the start of intervention development rather than applying to manualization efforts that build upon previous clinical practice interventions.

There are several important limitations and possible implementation challenges of art therapy manualization and the FRAME model. Although we have demonstrated how manualized art therapy protocols can be written such that they encourage flexibility and individualization, the inherent standardization of manualized protocols might limit an art therapist's ability to fully tailor their approach for each patient. We have yet to test how art therapists will engage with materials like the CDMG that both systematize and guide clinical decision making to respond to each Veteran's needs. Such testing will determine this approach's acceptability to art therapists when utilized with Veterans. The FRAME model presents an approach to manualizing art therapy protocols based on clinical practice and experience. However, implementation challenges include finding an interdisciplinary team of collaborators and the resources necessary to test a manualized protocol in a research study. If these challenges occur, we recommend that art therapists consider partnering with a peer or supervisor to develop their materials and consider utilizing larger art therapy networks through online forums, such as professional associations. If working with a small group, art therapists should consider recording observations of their work in art therapy sessions by taking careful notes on their process and generating a checklist based on their clinical observations to begin their manualization process. Once art therapists have established a protocol that they believe is effective, they should consider partnering with research teams to pilot and test their materials. Finally, if art therapists are working in settings where access to literature can be a barrier, they could consider comparing their approach to other treatments for benchmarking.

Creating a manualized CAT intervention can seem daunting, as it is difficult to standardize our work without losing its core creative and improvisational aspects. Such an approach is of great relevance to the CAT community as creating greater standardization in our work can support our learning and help us to build a firmer evidence base that supports our work.

From our experience, identifying the key components of treatment, while also outlining areas of individualization throughout the intervention, with concrete tools like the CDMG may balance standardization and flexibility. We recommend that creative arts therapists who seek to manualize their treatment identify the key aspects of their approach that must be delivered to everyone and provide guidance on how to adapt the intervention to incorporate client goals, support each person's unique background, and follow client curiosity, symbols, metaphors, and expression. Throughout ART-VET, we have highlighted Veteran and clinician choice as much as possible while maintaining the necessary structure and boundaries to deliver the intervention effectively.

Conclusion

Although CATs are often perceived as difficult to standardize due to their inherent flexible, creative, and responsive nature, our work in developing ART-VET demonstrates that it is possible to balance structure with clinical adaptability. The approach we described offers a framework for art therapy manual development that preserves individualization while promoting consistency in delivery. By outlining the FRAME model's step-by-step process for manualization, this work may inform future efforts to expand the evidence base for creative arts therapies. Leveraging the field's inherent strengths in creativity and flexibility, such approaches may support the development of scalable, evidence-based interventions that remain responsive to patient needs.

Disclaimer

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Declaration of Competing Interest

None to report.

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Data availability

No data was used for the research described in the article.

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